# Fulvestrant · injections every 4 weeks

NCCP schedule: 500 mg on Days 1 and 15 of the first 28-day cycle, then Day 1 of each 28-day cycle. This means Day 29 next, then every 28 days.

BASELINE → TREATMENT → FOLLOW-UP

## When checks happen

Read the row for your treatment setting. Columns show order, not equal lengths of time. Agree the due window, booking team, result reviewer and whom to contact if a check is missing.

| Check | Before starting | First weeks | During treatment | Longer-term review | Change / completion |
|---|---|---|---|---|---|
| **Bloods / symptom review** | FBC, renal and liver profile | Review tolerability and medicines | Further tests when clinically indicated | Review symptoms, adherence and results | Follow unresolved abnormalities |
| **ECG / echo** | No routine ECG / echo for this medicine alone | Investigate new cardiac symptoms | Further cardiac tests by risk / symptoms | Companion-drug cardiac rules remain separate | Follow abnormalities when indicated |
| **Bone density / DXA** | Assess personal fracture / osteoporosis risk | DXA only when indicated | No cycle-based DXA appointments | Repeat only according to the bone-risk plan | Continue bone care according to risk |
| **Advanced-disease response** | Document disease sites / baseline assessment | New symptoms can bring assessment forward | Generally every 2–4 months; individualise | Adapt interval and imaging to disease / benefit | Markers supplement, never replace, assessment |

THE WHOLE TREATMENT JOURNEY

# Fulvestrant · injections every 4 weeks: the whole treatment pathway

**Postmenopausal ER-positive locally advanced or metastatic breast cancer**

NCCP schedule: 500 mg on Days 1 and 15 of the first 28-day cycle, then Day 1 of each 28-day cycle. This means Day 29 next, then every 28 days.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | First weeks | During treatment | Longer-term review | Change / completion |
|---|---|---|---|---|---|
| **Advanced: medicine** NCCP 00361 | Confirm exact advanced indication | D1 and D15 loading visits | Next D29, then every 28 days | Review cancer control and tolerance | Agree next treatment / care |
| **Advanced: local treatment** ESMO_MBC2023 | Define any local-control / symptom need | Surgery / radiation only if indicated | Teams agree overlap or sequence | Review symptoms and local effects | Reassess goals if disease changes |
| **Safety checks** NCCP 00361 · SMPC | Baseline FBC, renal and liver profile | Review symptoms / interactions | Bloods when clinically indicated | Risk-based cardiac / other checks | Follow unresolved abnormalities |
| **Scans / bone health** NICE_NG101 · ESMO_EBC2024 · ESMO_MBC2023 | Baseline disease and fracture assessment | Confirm which setting-specific rows apply | Keep agreed scan / DXA appointments | Advanced response scans; risk-based bone care | Name who books and reviews results |

# What each check is for

### 1 Bloods and symptoms

Baseline FBC, renal and liver profile; later blood tests are clinically indicated, not automatic chemotherapy-cycle tests. At injection visits, review bleeding, bruising and injection / nerve symptoms.

Sources: NCCP 00361

### 2 Heart monitoring

There is no routine ECG or echo schedule for fulvestrant monotherapy. Monitoring required by a separately prescribed companion drug must be added from that regimen.

Sources: NCCP 00361 · SMPC

### 3 Bone health

Assess personal fracture and osteoporosis risk. Fulvestrant does not carry an automatic AI DXA timetable; agree testing and repeat intervals when indicated.

Sources: NICE_NG101

### 4 Cancer scans

Advanced: response assessment generally every 2–4 months, adapted to disease pace and symptoms; less often may suit indolent disease. The team selects imaging, not automatically CT chest/abdomen/pelvis. New symptoms can bring assessment forward.

Sources: ESMO_EBC2024 · ESMO_MBC2023

### 5 Markers

Tumour markers may supplement imaging and clinical assessment in advanced disease if they are informative, especially if elevated at baseline. They do not replace scans, and a marker change alone should not determine treatment failure. There is no universal fixed marker timetable.

Sources: ABC5_MARKERS

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.